Clinical Guideline for:
Use of Morphine/Midazolam sedation on ICU DURING COVID-19 PANDEMIC ONLY

Summary

- In the event that there is a shortage of propofol/alfentanil to sedate patients who require ventilation because they have COVID-19, infusions of morphine and midazolam provide an alternative choice
- Sedation with benzodiazepines is associated with the development of delirium, and therefore this regimen should be used as a last resort.

Key Points

- Morphine and midazolam are cardiovascularly stable drugs which can be used together to provide analgosedation for patients requiring ventilation.
- Though in widespread use in the late 1990s, their use has been superceded by drugs such as propofol and alfentanil which accumulate much less and are thought to result in less brain dysfunction.

Clinical use

- Morphine should be made up 50mg in 50mls 5% dextrose or Normal Saline.
- Midazolam should be made up 50mg in 50mls 5% dextrose or Normal Saline.
- Initial infusion rates should be in the order of 5mgs (=5ml) per hour.
- The drugs can be infused centrally or peripherally.
- Titrate to effect. Doses in excess of 10mg/hr of either drug should be avoided (and not necessary).
- If syringe drivers are in short supply, morphine and midazolam can be mixed in the same syringe.
- Daily sedation holds are a high priority in stable patients who are being sedated with M&M as prolonged use is associated with significant drug accumulation.
- If patients on sedation hold require re-sedating, restart at half previous hourly infusion rate.
- Morphine and midazolam infusions should be AVOIDED in patients with liver or renal dysfunction.

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